

Assessing the Medically Unwell Patient

The ABCDE Approach

Unlike <C>ABCDE, which is tailored towards managing the trauma patient, the ABCDE approach is for assessing medically unwell patients. The main difference is small C, which aims to look for signs of dehydration instead of blood loss.

A

Airway: Can the patient talk?

→ Yes? Move on to B

→ No?

- ☐ Check for airway obstruction
 - snoring, stridor, foreign body, secretions, vomit, blood
- ☐ Perform airway manoeuvres
 - head tilt / chin lift, jaw thrust
- ☐ If reduced GCS, consider airway adjuncts
 - OPA, NPA, LMA, iGel

B

Breathing:

- ☐ Observations
 - RR, sats
- ☐ RISE NTWELVE FALL
- ☐ O2 if sats <94%

C

Circulation:

- ☐ Observations
 - HR, BP
- ☐ Fluid assessment – check for signs of dehydration:
 - Cool hands? ➢ Reduced skin turgor over the clavicle?*
 - CRT > 2 seconds? ➢ Dry lips / mouth?
 - Weak, rapid pulse? ➢ Reduced urine output?

- ☐ Clinically dehydrated?
 - **Discuss with MO**

D

Disability:

- ☐ Observations
 - Blood glucose
- ☐ AVPU / GCS
- ☐ PEARL

E

Exposure:

- ☐ Observations
 - Temperature
- ☐ Abdominal exam
- ☐ Check calves
 - Pain, redness, swelling, warmth
- ☐ Check for rashes / bleeding / bruising

*Reduced Skin Turgor

Pinch the skin overlying the clavicle.

Let go – if the skin remains in the 'pinched' position, this is known as **reduced skin turgor**. It is a sign of dehydration.